

Consultation Details

Date: Friday, July 10, 2026
Doctor: Dr. Sarah Patel
Patient: Aashish Sah
Gender: Not Selected
Consultation ID: 6a4fec54ecbfc32d46a504ef

TRIAGE: HIGH

SOAP Clinical Note

S — Subjective

Patient reports sudden onset of 'crushing' substernal chest pain 45 minutes ago. Pain is radiating to left arm. Associated symptoms include diaphoresis and shortness of breath. Pain rating is 8/10.

O — Objective

Vitals: BP 148/92, HR 104, O2 Sat 94% on RA. Physical: Patient is diaphoretic, anxious, and in acute distress. Heart sounds tachycardic but regular, no murmurs.

A — Assessment

Acute Coronary Syndrome (ACS) - STEMI vs NSTEMI. Rule out myocardial infarction.

P — Plan

Immediate 12-lead EKG. Draw serial Troponins, CBC, BMP. Administer Aspirin 324mg chewable, Nitroglycerin 0.4mg SL. STAT Cardiology consult.

AI Triage Reasoning

Patient presents with crushing chest pain radiating to left arm, shortness of breath, and diaphoresis. These match indicators for Acute Coronary Syndrome (ACS).

Guideline Source:

Grounded in: AHA/ACC Chest Pain Evaluation Protocol - American Heart Association (AHA) and American College of Cardiology (ACC) Guidelines for the Evaluation and Diagnosis of Chest Pain (2021)

Consultation Transcript

Doctor: Patient: Doctor, I've been having this really intense pressure right in the center of my chest. It started about 45 minutes ago while I was just sitting at my desk.
Doctor: I understand. Can you describe the sensation? Is it sharp, or does it feel more like a weight or squeezing?
Patient: It's like a heavy weight, like someone is standing on me. It's actually starting to move down into my left arm now, and I'm feeling quite sweaty and short of breath.
Doctor: Are you experiencing any nausea or lightheadedness? I'm going to get the nursing team to start an EKG right away.
Patient: Yes, a bit of nausea. The pain is definitely getting worse. It's probably an 8 out of 10 now.

Clinician Approval

Approved By: Dr. Dr. Sarah Patel
Approved At: Friday, July 10, 2026

DISCLAIMER: This report was AI-assisted and has been reviewed and approved by a licensed physician. It is intended for clinical documentation purposes only and does not replace direct medical consultation.

